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Name: سميره عبدالملاك سعيد عطيه National ID: 25109010102223	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٥/٠٤ ٢٥١٠٩٠١٠١٠٢٢٢٣

ربة منزل

ارملة

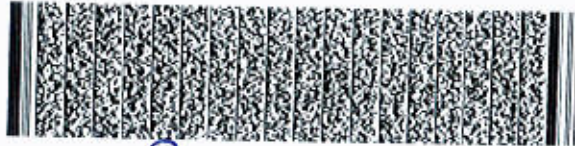
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البطاقة مارية حتى ٢٠٢٢/٠٤/٢٦

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د س

رئيسة البطنة



جمهورية فلسطين العربية
بطاقة تحقيق الشخصية

سميره

عبدالملاك سعيد عطيه

ش اولاد عبدالغزيز

الخصوص - القليوبية

١٩٥١/٠٩

٢٥١٠٩٠١٠١٠٢٢٢٣

LC4463019

7/8/26, 7:10 PM

code:02 -- Name_AMDT -- Name_E MDT

جوسيتاف
روسيمبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالصحة المصرية
Ministry of Health and Social Insurance

mm/dd/yyyy

MDT FORM

Name: <u>سيدة هداية محمد جابر</u>	MRN: <u>9737</u>
Age: <u>75 Y</u>	ID:
Menopausal Sta	Phone:
NRS SCORE:	Marital Status:
BMI:	Family History: <u>VE</u>
CO Morbidity: <u>GLUCOMA, BLIND</u>	Reason Of MDT:

Clinical Data:

R

L

BX: IDC GRADE 3
 ER: 8/8 PR: 8/8 HER2: NOT AMP KI67:

Sonomammography:

Mets Workup:

History:

Central portion of breast: Infiltrating duct carcinoma, NOS A 75 yrs old female pt, DM, Blind, -ve FH for malignancy, reported PS II. The condition started 2 months ago, when pt started to report to the relatives progression of a long standing Rt breast mass, associated with abdominal pain...pt sought medical advice at Baheya, where mammosonography revealed bilateral breast masses...biopsy was done from both revealing Rt IDC GIII ER8/8 PR 8/8 Her2 2+, Lt breast IDC GIII ER5/8 PR8/8 Her2 3+...CT CAP revealed ?? HFL, peritoneal masses, and ?? colonic mural thickening & 2 osseous lesions (on bone scan).

CT CAP 7-4-2026: Bilateral axillary LN. NO lung or liver lesions !! Just ant abdominal wall coarse calcific deposits.

PET CT 5/2026

LT PULMONARY NODULES AND OMENTOPERITONEAL DEPOSITS
 PT IS METASTATIC TO LUNG AND PERITONEAM FROM THE START

Recommendations:

MDT Outcome & Plan

For:

1. MRI abdomen.
2. +/- colonoscopy according to results of MRI
3. Echo & Labs
4. T.markers
5. Meanwhile, consider starting weekly Taxol (80 mg flat dose)/Herceptin



SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شيماء لاشين	د انسيه عيسوي				

8/7/2026

8/7/2026

Name: <u>سميره عبدالملاك سعيد عطيه</u>		
MR. : <u>9737</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The immunohistochemistry on the revised block demonstrated right-breast invasive carcinoma with estrogen receptor (ER) 8/8, progesterone receptor (PR) 5/8, HER2 score 2+ without amplification by SISH; the left breast showed ER 8/8, PR 8/8, HER2 score 3+. The patient reported headache and was scheduled for brain MRI and a guided biopsy from the hepatic focal lesion (HFL). A CT of the chest, abdomen, and pelvis performed on 7-4-2026 revealed bilateral axillary lymph nodes but no lung or liver lesions; only coarse calcific deposits were noted in the anterior abdominal wall. The case was forwarded for multidisciplinary team (MDT) review. A relative visit PET-CT with contrast in May 2026 showed multiple omental peritoneal deposits and a left basal pulmonary metastasis, indicating de novo metastatic breast cancer involving the peritoneum and lung. An MRI of the brain had not yet been performed; further MDT discussion was planned. The plan included MRI of the abdomen, evaluation of the colon, echocardiogram, laboratory studies, and tumor markers according to MRI results. In the interim, weekly Taxol 80 mg (flat dose) was to be initiated together with Herceptin. PET-CT of the breast and lymph nodes demonstrated pulmonary and omental disease; MRI of the right frontal bone revealed a lytic lesion with 60 % estimated tumor burden. The patient was to receive Taxol 80 mg weekly for nine weeks plus Herceptin. For week 2, the patient was to receive Taxol, carboplatin, and Herceptin after laboratory and MRI results; tumor markers were not repeated. The patient was scheduled for week 3 Taxol and carboplatin at the pharmacy. The pharmacy note for week 3 confirmed administration of Taxol 80 mg and carboplatin AUC 2. The patient, a 75-year-old blind individual with bilateral breast cancer metastatic to peritoneum, bone (skull), and lung, was described as right-sided IDC grade 3, ER 8/8, PR 5/8, HER2 2+ (non-amplified) and left-sided IDC grade 3, ER 8/8, PR 8/8, HER2 3+. PET-CT showed multiple peritoneal nodules (largest 21 × 27 mm, SUV 6.9) and a lower-lobe pulmonary nodule (12 × 8 mm, SUV 1). Brain MRI on 27 June demonstrated a left frontal lytic lesion (15 × 14 mm) with extra-osseous intracranial component indenting the dura. MRI of the abdomen was pending pending referral for severe headache and pain clinic consultation. The patient received week 4 Taxol 80 mg and a loading dose of Herceptin 710 mg. The pharmacy note indicated the patient was on Taxol, carboplatin, and Herceptin; a consultation with Dr. Kilany was attempted without response, and the plan was to continue Taxol and Herceptin loading dose this week with a follow-up consultation next week. For week 5 the patient received Taxol 80 mg; week 6 approval for zoledronic acid (Zometa) was pending and MRI of the abdomen remained pending. The		
تحريرا في ,		
	الوقت :	19-09-2026 التاريخ :
	خالد عبد العزيز محمد كمال	الطبيب المعالج :
يعتمد ,		

patient has completed five weeks of weekly Taxol therapy to date.

patient's name: SAMERA ABD EL MALAK SAEED A
Date: 2026-04-07
Referred by: Prof.Dr. CT CAP+ C
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported bilateral breast carcinoma ,

(I) Chest:

- ☐ Bilateral enlarged axillary lymph nodes , the largest on the right side measures 1.8 x 0.9 cm and on the left side 2x 1 cm
- ☐ No sizable hilar or mediastinal lymph nodes
- ☐ Clear both lung fields .no sizable pulmonary masses or consolidation patches
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.
- ☐ Heterogenous enlarged thyroid gland with coarse calcification

(II) Abdomen and pelvis:

- ☐ Anterior abdominal wall coarse calcific deposits .
- ☐ The liver is enlarged in size showing homogenous low CT density , no focal lesions
- ☐ Patent and homogeneously enhancing portal vein.
- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.

Patient's Name: SAMERA ABD EL MALAK SAEED A
Date: 2026-04-07
Referred by: Prof.Dr. CT CAP+ C
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- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.

٩٧٣٧

Name : Samira Abdelmalak Said

Date : 25-05-2026 12:10

Branch : Dokki

Medical No. : 382353

Ref.Physician Prof : Goustav Roussey Egypt

Whole-Body Bone Scintigraphy Report

CLINICAL HISTORY & INDICATION:

- 74-year-old female; staging for left breast carcinoma. Complains of diffuse bony aches.

TECHNIQUE

- Anterior and posterior whole-body scans were acquired 2 - 3 hours post injection of 20 mCi of Tc-99m MDP.

Comparison: Prior study dated March 15, 2026.

Findings (all stable versus prior study of March 15, 2026):

- A localized area of warm uptake at the left skull remains stationary without change in intensity or extent.
- Heterogeneous, mildly increased radiotracer uptake along the dorso-lumbar vertebrae, most pronounced at DV11, is consistent with advanced degenerative disease and unchanged.
- A focal, ill-defined area of mild equivocal uptake at the right iliac bone remains stable without interval progression.
- Bilaterally symmetric, moderately increased uptake over both knee joints, characteristic of active osteoarthritis, is also unchanged.

IMPRESSION

- No scintigraphic evidence of progressive metastatic disease. All previously indeterminate sites (left skull, right iliac) remain stable over 2 months, reducing suspicion for aggressive metastases.
- Indeterminate but stable foci - correlate with cross-sectional imaging (CT or MRI of skull and right ilium) for definitive characterization, given primary breast malignancy.
- Incidental degenerative changes - advanced spinal spondylosis (DV11) and bilateral knee osteoarthritis, unchanged.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

ECHO REPORT

NAME: *محمد بن علي*

DATE: 3 / 6 / 2026

ID NUMBER: /

SEX: AGE:

EF	60%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.5	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *N*

LV Wall Thickness *N*

LV Diastolic Function *GE*

SWMA *No. RWMA*

LA and Ao Root Dimensions *N*

Mitral Valve *Mild MR*

Aortic Valve *N*

Tricuspid Valve *Mild TR*

Pulmonary Valve

Right Side and Systolic Function *TAPSE = 2.3*

Intracardiac Masses and Effusion

DR.

[Signature]

Name: Samira Abdelmalk
 Referred by: Prof. Nagy Samy

Date: 20/5/2026
 ID: 30211006

F18-FDG PET/CT STUDY

Clinical History:

- 74 years old female patient presented with bilateral breasts lesions.

Reason for PET/CT study:

- Initial assessment.

Technique:

- Radiopharmaceutical: 18F FDG.
- Blood glucose at time of 18F-FDG injection: mg/dL.
- Time from 18F-FDG injection to scan: 60 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.

COMPARISON: No previous PET CT studies.

FINDINGS:

Patient Name	: الأستاذ / سمير عبد المائد سعيد	Request No.	: 13005613
Patient No.	: 25109010102223	Request Date	: 29/03/2026
Path No.	: SR-403/26	Verification Date	: 31/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 74 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Bilateral breast cancer.

Nature of Specimen: Referred blocks & slides (Bilateral Tru-cut biopsies).

Gross Examination:

- Two referred blocks labelled by (755/26) sublabelled (RT & LT) & Six referred slides including two H&E stained slides and four IHC stained slides for (ER, PR & HER2) for each side and one SISH slide for right side core biopsy.

Microscopic Examination:

1- Right breast mass:

- * Histological type: Invasive duct carcinoma.
 - * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
 - * DCIS: Minimal low grade ductal carcinoma in situ (solid pattern).
 - * Angioinvasion: Absent.
 - * Perineural invasion: Absent.
 - * Microcalcification: Present.
 - * IPT :
 - ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
 - Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Not amplified.
- (Average Her2 copy No.: (2.4), Her2/CEP17 ratio: (1.45)).

2- Left breast mass:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * DCIS: Intermediate & high grade ductal carcinoma in situ (Solid & papillary pattern).

Verified by :

Dr. Duaa Mohamed

Printing Date : 07/04/2026 17:19:06

Page 1 of 3

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / سمير عبد الملاك سعيد	Request No. : 13005613
Patient No. : 25109010102223	Request Date : 29/03/2026
Path No. : SR-403/26	Verification Date : 31/03/2026
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 74 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * IPT :
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

Diagnosis:

Bilateral breast cancer, Referred blocks & slides (Bilateral Tru-cut biopsies):

1- Right breast mass:

- Invasive duct carcinoma, Grade 3.
- Minimal low grade ductal carcinoma in situ (Solid pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
- (Average Her2 copy No.: (2.4), Her2/CEP17 ratio: (1.45)).

2- Left breast mass:

- Invasive duct carcinoma , Grade 3.
- Focal intermediate & high grade ductal carcinoma in situ (Papillary & solid pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3).

Verified by :

Dr.Duaa Mohamed

Printing Date : 07/04/2026 17:19:06

Page 2 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأمتلاءه / سميره عبد الملاك سعيد	Request No.	: 13005613
Patient No.	: 25109010102223	Request Date	: 29/03/2026
Path No.	: SR-403/26	Verification Date	: 31/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 74 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Bilateral bloody nipple discharge.

Nature of specimen:

- Right nipple discharge, Referred smear (Cytology).
- Left nipple discharge, Referred smear (Cytology).

Gross specimen:

- Only two smears were referred labelled by (512-19) and sublabelled as (RT & LT).

Microscopic examination:

- Cytological examination of the only received two smears of both (Right & left sides) revealed hypocellular smears entangling RBCs, lymphocytes & histiocytes amid focally hemorrhagic background. No cytological malignancy in smears examined.

Diagnosis

Bilateral nipple discharge, Both (Right & left) referred smears, Cytology:

- Bilateral findings are suggestive duct ectasia.

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

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Page 1 of 1

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